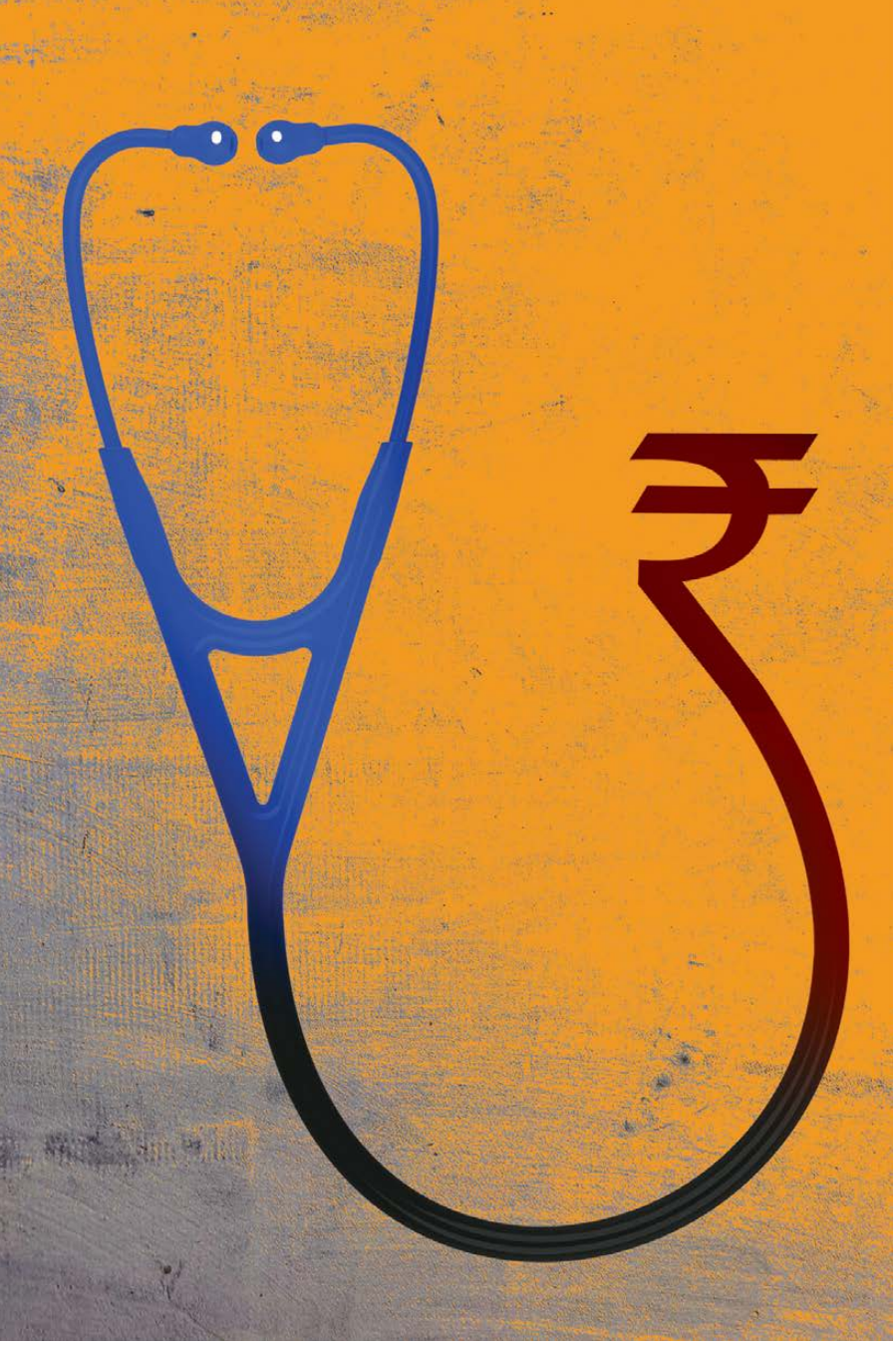


be strengthened. They must provide comprehensive and continuous care close to home, aided by point-of-care diagnostics, while being connected to more advanced healthcare facilities at secondary and tertiary levels through telehealth and emergency transport services. District and medical college hospitals must be upgraded to provide advanced care and also to serve as education and training hubs for medical and nursing students with allied health professionals of diverse categories.

Shortages in skilled health personnel are evident in all categories, with maldistribution across states and urban-rural divides exacerbating inequity. The needs of primary care can be addressed by training and deploying higher numbers of technology-enabled frontline workers (ASHAs, Auxiliary Nurse Midwives) whose upskilling can enable them to perform many functions currently assigned to doctors. Other categories of professionals too need to be trained in larger numbers, both to meet our health system needs and use our demographic advantage to supply a depleted global health workforce which is unable to serve ageing populations. We must promote our traditional systems of medicine, for integration into Indian and global health systems.

UHC has two measures: financial coverage and service coverage. Through adequate financial protection, we aim to reduce out-of-pocket and catastrophic expenditures (mostly related to outpatient care and hospitalisation respectively) and healthcare-induced impoverishment. But this becomes meaningful only when the healthcare that's needed is actually available and affordable. Therefore, India needs to cover gaps in both financial protection and service coverage. For affordable service coverage, we need to draw upon a more responsive public sector, a more responsible private sector and a more resourceful voluntary sector. Financial protection has to come predominantly from public sector provisioning and tax-funded government health insurance systems, with employer-provided and privately purchased health insurance playing a supplementary role.

A multi-sectoral approach: The health system must catalyse multi-sectoral policies that are aligned to public health goals—in spheres such as education, income security, nutrition-sensitive food and agriculture systems, environment, gender equity, urban design and transport. While promoting health and nutrition literacy in the population, it must shape public policies to curb aggressively advertised tobacco products, ultra-processed foods, sugar-sweetened beverages and alcohol.

India's large population is geographically, climatically, ethnically and culturally very diverse. The vast range of gene-environmental interactions, epigenetic changes and microbiome patterns in India can be studied to greatly enrich global science and fill gaps in our understanding of physical and mental health. Our pluralistic health systems can provide more complete responses to complex health problems through complementary approaches. Through frugal innovations and technological ingenuity, we can model new approaches to global health. ■

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Illustration by **NILANJAN DAS**